

1) Probable Diagnosis (Concise Summary)

The lab report indicates markedly elevated serum creatinine (6.94 mg/dL) and high urea levels (128 mg/dL),

suggesting acute or chronic renal dysfunction (kidney impairment). Mild hyponatremia (low sodium: 133

mMol/L) is also observed, which may be secondary to renal dysfunction or fluid imbalance.

Blood glucose is

within normal limits, ruling out hyperglycemia as a contributor.

2) Key Major Findings

- Serum Urea: 128 mg/dL (High) - above normal range (14.98-38.52)
- Serum Creatinine: 6.94 mg/dL (High) - above normal range (0.7-1.3)
- Sodium: 133 mMol/L (Low) - below normal range (136-145)
- Potassium: 3.90 mMol/L (Normal)
- Glucose (Fasting): 97 mg/dL (Normal)

3) Suggested Next Steps (Not Medical Advice)

- Confirmatory Tests:

- Repeat renal function test (RFT) and urinalysis to confirm elevated urea and creatinine.
- Ultrasound abdomen to assess kidney size and structure.
- eGFR (Estimated Glomerular Filtration Rate) to evaluate kidney function severity.

- Clinical Management:

- Review fluid intake and medications (especially nephrotoxic drugs).
- Consult a nephrologist for evaluation of possible renal insufficiency or failure.
- Monitor blood pressure, electrolyte balance, and urine output regularly.